3 The Legitimisation of Psychiatric Measures Against a Person's Will

3.1 Introduction

The treatment of people who were regarded as crazy, inappropriate or possessed probably involved certain forms of coercion in all phases of humanity. These people were excluded from social relationships, and they were locked up in various forms, just as strangers were regarded as barbarians and were fought against under certain circumstances. Historically, the aspect of involuntariness in dealing with people who displayed such "inadequate" behaviour can be seen as a constant. However, coercion has been legitimised in different ways over the last few centuries, which is linked to the development of psychiatric care, scientific ideas and social change in general.

Human rights have also been dealt with differently in different phases of the history of mental health care. As will be described in detail later in the book, two major phases can be recognised to date. The first phase lasted from the beginning of psychiatry as a science and as a medical discipline until well into the second half of the twentieth century. During this phase, which was largely characterised by institutions such as hospitals and asylums, human rights were largely disregarded. The institution-centred phase was followed by the "person-centred phase" from around the 1970s/1980s. Although human rights were now taken into account, the restriction of rights was justified on medical and ethical grounds.

3.2 Coercion in Psychiatric Care: The Institution-Centred Phase

Physical coercion and other measures against a person's will are therefore not only part of the present but also part of the past of psychiatry. The ancient Greek poet Aristophanes wrote that "madmen" were pelted with stones and Plato's "Laws" (section 934c7) contained a passage for dealing with "madness", which suggested

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that such people should be kept away from the public by their own family and that offences should be punished with fines [1]. Dealing with "madness" without coercion has never truly existed, at least in Europe, as can be surmised from these indications. The philosopher Michel Foucault characterised the institutionalised beginning of psychiatry in the seventeenth century as the "great confinement" [2]. Later historical research confirmed the aspect of confinement in psychiatry but shifted it to the end of the eighteenth century and the beginning of the nineteenth century and established a link with the Enlightenment. "(T)he history of psychiatry began as a history of custodial asylum, institutions to confine raging individuals who were dangerous to themselves and a nuisance to others" ([3]: 7). In custodial asylums, the idea of protection takes precedence over treatment. Even then, however, according to the historian Edward Shorter, the idea behind the "great confinement" was that placement in asylums against one's will could have a curative effect.

In addition to the history of psychiatry, the history of law is also highly important here. In the transition to the early modern period, the idea of a benevolent state that was allowed to make decisions about its subjects in certain situations prevailed in absolutist states. In the Anglo–Saxon legal tradition, this principle from Roman law was referred to as *parens patriae* [4]. The respective ruler was identical to the state and, according to this legal doctrine, also acted as a parent. As the terminology suggests, the *parens patriae* doctrine was predominantly applied in legal matters concerning the welfare of children. However, the principle of the benevolent state was also applied to people who were considered "mentally ill" [5]. In the United States, for example, it was regarded as a legal principle until well into the 1970s [6]. Although the principle was not referred to as such on the European continent, it was handled in a similar way under the concept of state "care" [7]. Involuntary placements are still called "welfare placements" (*fürsorgerische Unterbringungen*) in Switzerland today.

However, in nineteenth-century England, the nuisance to other people mentioned by Edward Shorter was possibly one of the main factors behind the establishment of *madhouses*, as they were known in the British Isles at the time. These were mainly privately funded institutions where wealthy families could house relatives who appeared to display unconventional behaviour and were therefore seen as a problem for the family's reputation. This unconventional behaviour was not only associated with what is now classified as mental illness but also involved socially offensive behaviour such as a liaison with a person from a lower social class or politically unpopular statements.

A prominent case in today's historical literature but also in the medical literature of the time, is known as the *Lanchester Case.* One of the best-known medical journals of the time, the *British Medical Journal*, reported that Edith Lanchester was institutionalised by a doctor at the instigation of her father and brothers [8]. The reasons given by the doctor for the compulsory measure were as follows: Miss Lanchester had started an "illicit relationship" with a man who was clearly below her social status and whom she did not intend to marry. She had also always been eccentric and had recently started to espouse socialist ideas. She was admitted to the asylum because she wanted to commit "social suicide" and was diagnosed with

"monomania" on the subject of marriage. Both the family doctor who was called in later and the medical specialists in the institution confirmed the hospitalisation, which was eventually rejected by the *Lunacy Commission.*

Other people were less fortunate. Here is an example that symbolises the fate of hundreds of thousands of people whose names have been forgotten: the writer Elsa Asenjieff was involuntarily admitted on the basis of the diagnosis "troublemaker delusion" (*Querulantenwahn*) in Leipzig, Germany, in the 1920s. She died in 1941 after several decades in hospitals and asylums. Medical-historical research has identified social background as a decisive factor in her tragic life story in Saxon psychiatry: "...as a single woman whose behaviour did not conform to social and gender norms and who had no family support, she already lived increasingly in poverty and isolation in the war-damaged young Weimar Republic" ([9]: 91).

This mixture of medical and social criteria for hospitalisation, which was generally involuntary, was applied in many countries until well into the second half of the twentieth century. At the end of the 1950s, an American psychiatrist described the traditional indications as follows:

> (1) behaviour which endangers the patient's life and (2) behaviour which the patient's environment will no longer tolerate. These classic indications for hospitalisation characteristics in common—both usually arise out of emergency situations, both are manifestations of severe degrees of mental illness. In either case, the patient does not seek hospitalisation himself—others seek it for him. And in either case the prescription for hospitalisation is usually made by the family, friends, judges, police—and of particular interest historically—rarely by a psychiatrist. The psychiatrist seldom disagrees with the lay prescription because the severity of the patient's illness and his need for hospital care are usually obvious. ([10]: 207)

Psychiatric institutions or asylums were established in almost all European countries during the nineteenth century. Those who were admitted often had to spend many years there. The conditions in most institutions are inhumane and socially and hygienically catastrophic. A few years after the end of the institution-centred phase, two German psychiatrists described this phase as a "...scandalous practice of asylums for the mentally ill and mentally handicapped in prison-like, oversised detention facilities that inevitably produced hospitalisation defects, whose equipment was mostly unworthy and whose staffing just allowed for supervision but no treatment..." ([11]: 588).

The historian Edward Shorter quotes a psychiatrist who worked at the Maudsley Hospital in London in the 1930s as saying, "Life was not quite so pleasant for the patients. Confined to locked wards with an enclosed garden for exercise, patients whose stormy illness had blown itself out, leaving only residual symptoms, would remain year after year, unoccupied, deprived of incentive or responsibility of self-determination, getting more and more fixed in the straitjacket of an unchanging daily routine" ([3]: 65f).

Most "inmates", as they were known at the time, had no rights with regard to therapy. They were, as the sociologist and historian Andrew Scull wrote, "...deprived of their status as moral actors, and presumed by virtue of their mental state to lack

the capacity to make informed choice for themselves, patients were mostly unable to resist those who controlled their very existence…" ([12]: 308). Thus, sometimes brutal physical interventions were carried out against the will of those affected, from insulin shocks to surgical interventions in the brain (e.g. lobotomies) to electroconvulsive therapy, which were only carried out later under anaesthesia. The lack of rights was also one of the prerequisites for the murder of thousands of people with mental, cognitive and physical disabilities during National Socialism in Germany.

The associated ideas about people with mental health problems also dominated in former Eastern Germany. In 1979, Ehring Lange, one of the country's leading psychiatrists at the time, succinctly summarised the perceptions that still had to be overcome in the 1960s. A critical approach was needed:

> with the view that schizophrenic illnesses inevitably and irreversibly lead to a defect and that their core group should not be given special intensive treatment, with the view that a binding prognosis can be inferred from a cross-sectionally recorded symptomatology—which then predetermines the value or lack of value of therapy and rehabilitation, with the view that work therapy is a means of filling unused daytime for chronic institutionalised patients, but is superfluous for acutely ill patients in clinical treatment, with the view that the mentally ill person is fundamentally unpredictable, dangerous to himself and to the community owing to the inherent laws of the illness and therefore in principle requires closed accommodation and segregation from society. [13]

Thus, from today's perspective, abstruse judgements were not limited to the former GDR. "Mentally ill" people almost always act under pathological influences; therefore, there was no "inner freedom", and restrictions on movement were unproblematic, according to a medical opinion on the introduction of mental health laws in West Germany in the 1950s ([14]: 24). As part of the introduction of a legal framework for hospitalisation, there was also a discussion between legal and psychiatric experts about the involvement of courts in hospitalisation proceedings. On the medical side, there were fears of a restriction of medical therapeutic sovereignty and a strain on the therapeutic relationship ([14]: 24).

Many institutions in Eastern and Western Europe, which saw themselves as hospitals over the course of time, also took part in drug trials in which most patients were probably only asked for verbal consent at best; informal coercion can usually also be assumed in these cases. Until the 1960s, it was not common practice in all medical research to ask affected patients for consent [15]. Historical researchers in Switzerland documented these procedures for many psychiatric hospitals until well into the 1970s. These experiments with unauthorised medication resulted in numerous deaths, most of which were not documented or declared to relatives. The fact that negligent handling of consents also occurred among medical staff has become known in the context of the LSD trials at the Burghölzli University Hospital in Zurich. In 1947, staff were unknowingly given the substance in coffee [16]; in the case of patients on whom the "fantastic drug", as it was called at the time, was also tried, the corresponding article noted that they or their relatives had been asked for consent [17].

In general, medication was almost exclusively used as a treatment. Until the 1990s, for example, pronounced high-dose neuroleptic therapy was used [18], which was associated with considerable health problems for many patients. Symptoms were primarily suppressed, as this high-dose therapy already implies. Psychodynamic and later biomedical disease models dominated therapeutic reasoning. Surveillance and exclusion took priority over reintegration; there was no real rehabilitation. At best, those affected were transferred to other institutions, such as care homes and sheltered workshops. Physical needs were satisfied to some extent but slightly more than that. In professional training and research, people with "mental illnesses" were predominantly objects.

It was only from the 1950s in the United States and from the 1960s in Europe that conditions changed, albeit very slowly, as a report on a reform project in a state hospital in the United States from 1968 makes clear. It is also interesting to see how staff and the people locked up there treated each other: "Walls were painted, pictures and calendars had been introduced; and patients were given measures of dormitory and bathroom privacy. However, there was little concomitant change in the psychological architecture of the wards. Patients continued to fear staff, while staff continued to fear patients; patients retreated from the staff, who as often retreated from them. The staff felt hopeless about the patients, the patients felt hopeless about the staff" ([19]: 591f).

The legitimisation of coercion in this institution-centred phase was based on two intertwined lines of argument, which were, however, criticised even in legal circles from at least the 1960s onwards owing to their vague criteria and very flexible interpretations [20]. On the one hand, reference was made to the maintenance of public order. Particularly at the beginning of the phase from the end of the eighteenth century onwards, little emphasis was placed on the "state of mind"; public order was far more important. This then changed with the development of psychiatry as a medical discipline. In addition to the assumption that "mentally ill" people were "dangerous" and the resulting need for institutionalisation, a further justification was provided that is abstruse from today's perspective but still applies in certain variations to the present day: "mentally ill" people are not truly free due to their illness, so the deprivation of liberty through institutionalisation cannot be seen as such [14]. It is therefore hardly surprising that the persons affected by institutionalisation were generally not granted any rights. They were usually unable to refuse a doctor's order, were not given any information about the results of medical examinations and often did not know what medication they had to take [21].

3.3 From the Institution-Centred Phase to the Person-Centred Phase

"Negotiating instead of treating" (*Verhandeln statt Behandeln*) was the German-language motto of reform psychiatry, which at the same time described the core of care that was later referred to as person-centred support. The slogan was coined by the reform psychiatrist Asmus Finzen in the early 1990s, who wanted to clarify how

social and community psychiatry differed from the institutional psychiatry of the time. As Finzen later wrote, it was the tangible "often experienced paternalism conducted by doctors" that motivated both service users and relatives to reject the traditional care system, which was sometimes associated with considerable coercion and human rights violations [22].

However, the 'person-centred approach' was not new in the 1990s when psychiatry discovered it. The basic idea came from one of the central figures in the history of psychotherapy, Carl Rogers, and was formulated back in the 1950s. According to Rogers, person-centredness, which is also known as client-centredness, has three essential characteristics of a therapeutic relationship [23]. The first is authenticity. This means, for example, dispensing with a professional façade and creating a congruent situation in which nobody pretends. The second characteristic is the creation of an accepting attitude that is non-judgemental and can therefore bring about change on the part of the client. Third, it is about empathic understanding, which is achieved through active listening, for example.

The concept of "patient-centred medicine" also dates back to the 1950s and 1960s and was propagated by the psychoanalyst Michael Balint as a contrast to "disease-centred medicine" [24]. According to this idea, the person to be treated should no longer be seen as just a person with a broken bone but as a "unique human being" who should receive an *overall diagnosis* and be treated accordingly.

Patient-centred medicine later became the core quality criterion for good medical care as a whole. In its 2001 report on quality deficiencies in healthcare, the US Institute of Medicine declared patient-centeredness to be one of the main objectives, along with other characteristics such as equity, safety, speed, effectiveness and efficiency [25]. According to the report, patient-centredness, in turn, has the following characteristics: (1) respect for patients' values and preferences, (2) coordination and integration of care, (3) information, communication and education, (4) physical well-being, (5) emotional support and (6) involvement of family and friends.

Over time, the aforementioned characteristics of patient- or person-centredness have increasingly been translated into skills that healthcare professionals such as doctors, nurses or therapists should learn as part of their training. Numerous studies have been conducted in this context, which have shown, on the one hand, that the skills can certainly be taught and applied by professionals but, on the other hand, that the effects on healthcare users cannot be measured so clearly, as seen in a large review from the Cochrane Initiative [26].

Another development that was already present in Balint's "patient-centred medicine" is the conceptual focus on the "person" and no longer on the "patient". This was intended to express that the person to be treated should be considered "holistically" and should not be reduced to the status of a patient. Person-centred care, according to one of the central theoretical contributions, is the "antithesis to reductionism" ([27]: 249). Methodologically, this involves two important elements: first, the story that the sick person has to tell and, second, *shared decision-making*. The latter has become increasingly central to person-centred care in recent years. The idea is that professionals and the person concerned, as well as other stakeholders

such as relatives, should decide together for the benefit of all which treatment methods should be used.

However, what exactly was the difference between the community psychiatry of the reform years and the former institutional psychiatry? The Austrian sociologist Rudolf Forster summarised the reform efforts in the mid-1990s and described the project as follows [28]: Instead of a purely organic concept of illness, a multifactorial model was used that also contained psychosocial components. The aim was social integration instead of hospitalisation. Instead of medical dominance, a multiprofessional team was to advise on treatment. The legal basis should no longer be predominantly coercion but rather voluntariness. Instead of paternalistic directives on treatment, it was a partnership-based consultation that allowed the patients to decide—in other words, person-centredness.

Rudolf Forster came to a sobering conclusion at the end of his analysis. "The gap between claim and announcement on the one hand and the results and effects on the other is strikingly large in many reform projects" ([28]: 249). The reason for this, according to Forster, was quite clear: "Psychiatry has become above all a relatively *conventional medical discipline*" ([28]: 249; emphasis in original). Although psychiatry had changed in various respects and brought about many changes, medical dominance had not been fundamentally broken but had been consolidated by the fact that psychiatry had become increasingly similar to conventional medicine.

The integration of psychiatry into general medicine was not an accident of the reforms; it was the declared intention. In addition to the equalisation of the physically and mentally ill (as it was called at the time), the establishment of psychiatric wards in general hospitals in various countries, such as Italy and Germany, symbolised this integration. The aim was to change the special status and stigmatisation of psychiatric treatment through normalisation in medicine. However, psychiatry, and thus also social psychiatry, also aimed to normalise medicine conceptually, which meant, for example, a clear preference for pharmacotherapy over talk therapy [29]. It also meant a diagnostic model that described "mental illnesses" as part of general illnesses, for example, in the International Classification of Diseases (ICD), developed by the World Health Organisation. The ICD is used almost worldwide—with the exception of the United States—and mental disorders make up a central chapter.

Overall, it was an endeavour to treat less, i.e. to know and decide paternalistically what is good for the user, and to negotiate more, i.e. to attempt to arrive at a joint approach and make decisions "on an equal footing". The core of the endeavours, for example, in Germany, was "... the consistent orientation towards the individual support needs of the mentally ill person" ([30]: 8).

In principle, the concepts in both German-speaking and English-speaking countries were relatively unanimous. Person-centredness primarily meant finding out from service users what help they needed and meeting these needs as part of a coordinated help system involving community-based services. Other concepts from the early days of the person-centred approach envisaged a *Person-centred Integrative* Diagnosis (PID) for this purpose [31]. The PID was to cover the following aspects: positive and negative health aspects, which were to be assessed within the context of the person's life, social functions, protective factors, quality of life and

information on dignity, values and motivations. However, all of this should be done within the framework of conventional classification systems such as the ICD. With reference to medical classification systems and treatment, it was made clear that person-centredness was still to be seen in a medical-psychiatric context.

In the 2010s, the concept of person-centredness underwent a change. With the recovery approach, the service user's perspective was generally incorporated more strongly into care. However, it is still unclear whether the combination of person-centredness and recovery would actually mean a fundamental change or whether recovery could be seamlessly integrated into person-centredness. There is still no consensus among experts on this topic. For example, it is argued that person-centredness should serve to support service users in the sense of recovery to lead a meaningful life [32]. Others see person-centredness together with recovery but also with the human rights approach as conceptual innovators in psychiatric care [33]. Third parties, in turn, see person-centredness as a culturally enriched, relationship- and recovery-oriented approach [34].

Other perspectives emphasise important differences. The recovery approach, which is already considered quasi-mainstream today, was notably not developed by clinicians but by those affected and those using it as a perspective that can run counter to medical understanding [35]. This is particularly the case when "illness" is no longer the central focus but rather the goal of leading a meaningful life despite or because of the illness, which does not necessarily mean adhering to psychiatric treatment methods and pharmacotherapy in particular. Recovery also favours support methods that are not considered evidence-based even in social psychiatry, such as the Open Dialogue method. Despite all the convergences with the person-centred approach, recovery is an approach that aims to achieve changes that are "consumer-driven" ([35]: 108). Overall, considerable ambivalence can be observed in current psychiatric care in this context. On the one hand, evidence-based care is upheld, while on the other hand, recovery and other concepts are propagated, at least programmatically.

3.4 The Legitimisation of Coercion in the Person-Centred Phase

One of the ambiguities of the concept of person-centredness is that it can go hand in hand with both recovery orientation and the perspective that healthcare professionals know what is good for the user. It is therefore not surprising that in the relevant literature, person- or patient-centredness is seen as a concept that should help to ensure that medication is taken consistently (e.g. in the adherence approach) [36]. It is just another step in a dubious direction when even coercive measures are discussed as potentially person-centred [37]. The fact that this abandons the idea of "equal footing" and shared decision-making on difficult issues in the context of treatment and care does not seem to matter.

Although a certain inconsistency or even abstruseness must be perceived here, it should be noted that this does not necessarily contradict conventional medical or

psychiatric ethics. Person-centredness—as just described—also has a supposedly objectifiable side, and in conventional medical ethics, decisions are generally not made from the perspective of the person concerned but from an external perspective "for the good of the person concerned". This is also understandable insofar as many of the ethical situations to be discussed in acute medicine are based on the assumption that the persons concerned are unconscious and would consent to the measures. In legal terms, this is conceptualised as "natural will" [38].

This is somewhat different in psychiatric settings, as the people concerned are generally not unconscious. Historically, the views of the people responsible were certainly similar. Until the end of the last century, it was hardly questioned whether coercion could not be for the benefit of the person, especially as there was virtually no empirical research on this question in addition to the widespread paternalistic views. This was not to change until the mid-1990s, when ethical and professional discussions became much more important [39]. Over time, however, the conditions under which it could be ethically legitimised to treat people psychiatrically against their will were increasingly discussed. Is it sufficient to establish a medical need, or must the illness be associated with a risk to oneself, and what about risks to other people? Furthermore, what type of therapy may be used? Is electroconvulsive therapy also permitted against the patient's will, as was common practice for decades in the second half of the last century? All these issues, which were previously practised more or less unquestionably, have now been ethically and empirically analysed [40].

Furthermore, many countries have developed ethical and practical guidelines for dealing with suicidal behaviour and risk behaviour to others in psychiatric settings. There have even been attempts to harmonise this within a European framework [41].

In addition to professional and ethical aspects, legal considerations naturally play a central role in the legitimisation of coercion in psychiatric care. Both national and international courts and legal conventions have been increasingly concerned with this issue since the 1970s. According to a survey on the ethical and legal aspects of compulsory admission in 40 European countries, psychiatric coercion is currently legitimised by the combination of the presence of a "mental illness" and the risk to one's own health or the health of others [42]. The need for treatment as determined by medical professionals and significantly impaired judgement alone can also be used as justification in individual countries [43]. Furthermore, it is generally recognised that the measure should be a therapeutically effective intervention, that it should be used as a *last resort* and that it should be the *least restrictive alternative*. Overall, it is assumed that the measure is in the best interests of the person concerned.

The German Ethics Council has addressed this issue precisely in a comprehensive paper [44]. The document not only covers psychiatric issues in a narrower sense but also addresses other issues, such as how to regard age-related restrictions or involuntary measures in children and adolescents. The Ethics Council considers many of the measures used in psychiatry and other areas of healthcare to be ethically problematic but rejects a general renunciation of coercion in "professional care relationships", as one member of the Ethics Council explained: "In the view of the Ethics Council, coercive measures can be justified only if the person expresses wishes and needs for actions or the refusal of actions that threaten to seriously harm

him or her but is not capable of making freely responsible decisions. A general rejection of all forms of coercive measures would lead to many vulnerable people being left to their fate. This is out of the question for the German Ethics Council" ([45]: 177).

Interestingly, the Ethics Council's opinion is that it only analyses coercion against the background of potential or actual harm to the individual. Dangers to other people were expressly not considered, although it is unclear for what reasons. One conceivable justification would be that medical intervention is not necessarily required to avert danger to other persons but that this would be possible in the context of police or judicial activities. However, no information was provided as to whether this justification applies, and it is therefore speculation.

In this context, the German Ethics Council is moving within the general framework of the ethical justification of coercion in psychiatry, as a systematic analysis of the arguments commonly used internationally has recently shown [46]. The ethical argumentation in favour of coercive measures in certain circumstances provides for the following aspects to be taken into account: autonomy and integrity are threatened by coercion but can also be restored by such a measure. The same applies to human dignity. Coercion can be used for the benefit of the person concerned (benevolence) and serves to avoid harm (non-maleficence). Coercion can also be fair insofar as individual and social interests are weighed. It can ultimately serve the safety of others as well as oneself and thus also be for the good of the greater whole.

In practice, these arguments also play a role. In general, in areas where coercion is used in psychiatry, the necessity and ultimately positive outcome of such measures are assumed. In addition, it is often argued that measures taken against a person's will also have therapeutic effects, such as promoting "insight into the illness" and compliance with treatment in general, as well as promoting the recovery process [47]. In addition, coercion serves to restore social order. However, in practice, such measures may jeopardise the therapeutic relationship [48, 49]. These negative consequences, which are also reflected in care settings, testify to ambivalence in dealing with coercive measures. Nevertheless, in practice, the arguments of necessity and the ultimate benefit for a person's well-being prevail [50].

The research literature on this topic draws attention to considerable social change that has also been reflected in care practice. Both in general psychiatric care and in forensic practice in particular, there has been a certain shift in many Western countries from primarily therapeutic aspects to safety aspects as well as risk assessment and risk avoidance [51, 52]. In this respect, it is hardly surprising that these aspects still play a strong role in the legitimisation of coercion in psychiatry today.

If one disregards the numerous country-specific characteristics in dealing with psychiatric coercion [42], the following schematic sequence emerges for the legitimisation of coercion in psychiatric contexts (Fig. 3.1).

The existence of a "mental disorder" with perceived impaired mental capacity or competence, combined with harm that has already occurred or a suspected risk to oneself or other persons, therefore justifies a coercive measure if it is used as a last resort and as the least restrictive measure possible. The overall measure is—so the assumption—in the interest and for the benefit of the person affected by the coercive

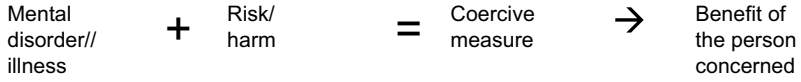

Fig. 3.1 Justification scheme for psychiatric coercive measures

measure, who would approve of the measure on the assumption of mental recovery or in hindsight. It is also assumed that the coercive measure is intended to restore the autonomy of the person concerned, which is considered to be significantly impaired by the mental disorder. This all presupposes that the therapy associated with coercion is effective and produces the desired results.

3.5 Conclusion: The Legitimisation of Coercion in Psychiatry: The Clinical-Ethical-Legal Stalemate

The legitimisation of coercion in psychiatry has changed considerably over the last few centuries, as the previous explanations have made clear. Whereas previously, the focus was on segregation and supposed dangerousness, this became much more medicalised and psychologised over the course of the twentieth century. Increasing emphasis has been placed on individual characteristics and backgrounds. However, this has not led to the abandonment of measures against the will of the person, as one might have assumed if the individuality of the person was better understood.

Medicalisation and psychologisation may have had the exact opposite effect. By focusing on the individual person, it may have been possible to better describe the risks that the person could pose. The same applies to the ethical discussion in general, which has manoeuvred itself into a dead end. The same ethical arguments, such as human dignity and the welfare of the person, can be used to argue both in favour of abolishing coercion and in favour of retaining it. Even clinical arguments can be made in favour of both the retention and abolition of coercion. If one side can legitimately argue that mental health problems can impair judgement, the other side can just as legitimately point out that the paternalistic treatment associated with coercion does not lead to the person's mental health and recovery. On a political and legal level, this situation has already been described as the Geneva impasse [53]. This refers to the fact that within the United Nations and the World Health Organisation (WHO), there are both conventions that legitimise coercion and those that attempt to abolish measures against a person's will. This means that even at the level of human rights, both sides are equipped with good arguments [54].

In this respect, ethical, legal, human rights and even clinical arguments do not truly help in the end. For an empirically orientated scientist such as me, part of the discussion is difficult to understand anyway, as the relevant empirical studies are often ignored. This will now be made up for. The question of whether coercion in psychiatry can be legitimised thus becomes an empirical question. It remains to be seen what studies and data are available to answer the question of whether coercion

is actually for the benefit of the person concerned, for example. These empirical questions will be answered in the following chapter.

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